

1. A 27-year-old man comes to student health services because of a 6-week history of increasingly severe insomnia and persistent fatigue. During this time, he also has had feelings of guilt, emptiness, hopelessness, and profound sadness. He is a graduate student and has withdrawn from most social activity. He says that during the past 4 years he has heard "Sauron" from a fantasy novel instructing him to renounce his family and forswear all sexual activity. He notes that he has had two episodes of depressed mood during the past 2 years. He began sertraline therapy during the second episode and had improvement in his mood; he discontinued the therapy 6 months ago because he was feeling well apart from intermittent hallucinations. He has no other history of serious illness and currently takes no medications. Physical examination shows no abnormalities. On mental status examination, he has a depressed mood and restricted affect. His thought processes are mildly tangential. He does not appear to be responding to internal stimuli. He has not had suicidal ideation. Which of the following is the most likely diagnosis?

- ☐ A) Dysthymic disorder
- ☐ B) Major depressive disorder with psychotic features
- ☐ C) Schizoaffective disorder
- ☐ D) Schizophrenia
- ☐ E) Schizotypal personality disorder



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2. An 8-year-old boy is brought to the physician for evaluation of difficulty reading. His teacher reports that he cannot keep up with his classmates. His mother reports that he is generally well behaved but frequently flaps his hands when he is nervous. Physical examination shows a high forehead, prominent supraorbital ridges, protruding ears, and a prominent jaw. Psychoeducational testing of the patient shows an IQ of 65. Academic achievement testing is consistent with his IQ. Which of the following is the most likely explanation for these findings?

- ☐ A) Congenital rubella
- ☐ B) Decreased numbers of trinucleotide repeats of the X chromosome
- ☐ C) Decreased serum hypoxanthine phosphoribosyltransferase activity
- ☐ D) Fragile site on the X chromosome
- ☐ E) Increased expression of the homeobox A1 (*HOXA1*) gene
- ☐ F) Increased serum concentrations of phenylpyruvic acid
- ☐ G) Increased serum concentrations of pyruvate
- ☐ H) Trisomy of chromosome 21
- ☐ I) XXY karyotype



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3. A 28-year-old man is brought to the emergency department because of flu-like symptoms and cough productive of clear sputum for 24 hours. He has been hospitalized eight times over the past 5 years because of bizarre delusions and hallucinations during which a voice tells him what to do. He has a 9-year history of type 1 diabetes mellitus. In addition to insulin, he is receiving clozapine therapy (400 mg daily) for his psychotic condition. During mental status examination, he is hallucinating and says that he is being pursued by special agents of the government. Leukocyte count is $2750/\text{mm}^3$, and serum glucose concentration is 140 mg/dL. Urinalysis is within normal limits. Which of the following is the most appropriate next step in management?

- ☐ A) Add lithium carbonate therapy
- ☐ B) Add penicillin therapy
- ☐ C) Decrease the dose of clozapine
- ☐ D) Discontinue clozapine therapy
- ☐ E) Increase the dose of clozapine



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4. A 15-year-old boy is brought to the physician by his parents because "he does whatever he wants and comes and goes as he pleases." They report that he always has been challenging to manage and has not responded to disciplinary measures since he was 10 years of age. During the past 2 years, his behavior has become worse; he has been arrested twice in the past month, once for assault and once for vandalism. He is currently on probation. His parents state that they have found him intoxicated in his room on several occasions, and they recently found marijuana paraphernalia in his room. Today, the patient is sullen and minimally cooperative. He says he is frequently irritable and short tempered. His clothes smell of cigarette smoke. Physical examination shows several tattoos over the chest and upper extremities and several body piercings; no other abnormalities are noted. Mental status examination shows an irritable mood and poor eye contact. Which of the following is the most likely diagnosis?

- ☐ A) Antisocial personality disorder
- ☐ B) Bipolar disorder
- ☐ C) Conduct disorder
- ☐ D) Dysthymic disorder
- ☐ E) Intermittent explosive disorder
- ☐ F) Oppositional defiant disorder



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5. A 16-year-old boy comes to the physician because of a 1-year history of progressive headaches that have caused him to miss several days of school. They now occur daily and are always present on awakening in the morning. He describes the headaches as a diffuse, constant pressure that occasionally throbs. He feels better after taking acetaminophen and resting in front of the television for 10 to 20 minutes. He has not noticed any particular events that cause the headaches, and he says that they are not exacerbated by light, sound, coughing, or straining. He has not had nausea or vomiting. Two months ago, he sustained a whiplash injury in a motor vehicle collision. His parents say that he snores loudly. His mother and maternal aunt have migraines, and his father has major depressive disorder. The patient is 168 cm (5 ft 6 in) tall and weighs 86 kg (189 lb); BMI is 31 kg/m². He breathes through his mouth. Respirations are 20/min. Examination shows 3 to 5 beats of jerk nystagmus with far lateral gaze bilaterally. Which of the following is the most likely cause of this patient's headaches?

- ☐ A) Hydrocephalus
- ☐ B) Idiopathic intracranial hypertension
- ☐ C) Major depressive disorder
- ☐ D) Migraine
- ☐ E) School-related anxiety
- ☐ F) Sleep-related hypoventilation



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6. A 24-year-old woman reports a 3-week history of recurrent fevers. She has a 4-year history of major depressive disorder. Her temperature is 42°C (107.6°F), pulse is 80/min, respirations are 14/min, and blood pressure is 110/70 mm Hg. Physical examination is otherwise normal. Her leukocyte count is 7500/mm³ with a normal differential. Liver function tests and serum creatine kinase activity are normal. Which of the following is the most appropriate next step in patient care?

- ☐ A) Administer broad-spectrum antibiotics
- ☐ B) Administer corticosteroids
- ☐ C) Obtain a rectal temperature under supervision
- ☐ D) Order a bone marrow biopsy
- ☐ E) Order a CT scan of the abdomen



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7. A 37-year-old man comes to the physician because of a 3-week history of fatigue, insomnia, and anxiety. His divorce was finalized 1 month ago, ending his 10-year marriage to his wife, who he learned was having an affair with his best friend. He feels anger toward his ex-wife and his friend, but he says, "I know what they did was wrong and that I am a good person who got hurt by them." He has no regrets about the divorce because he believes that he never would have been able to trust her again. He is relieved that they have no children. He often thinks about whether he will remarry, which causes anxiety and prevents him from sleeping. He has had no change in appetite. He works as a middle school teacher and has missed several days of school because of tiredness. His mind wanders when he watches television or reads at home. He has decreased energy but still enjoys activities with family and friends. He has no history of medical or psychiatric illness and takes no medications. Vital signs are within normal limits. Physical examination shows no abnormalities. On mental status examination, he is cooperative and engaging and exhibits a full range of affect. He says that his mood is okay. His thoughts are organized. There is no evidence of suicidal or homicidal ideation. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Fluoxetine
- ☐ B) Lithium carbonate
- ☐ C) Methylphenidate
- ☐ D) Olanzapine
- ☐ E) Zolpidem



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8. One month after undergoing liver transplantation, a 47-year-old woman is admitted to the hospital for treatment of organ rejection. Twenty-four hours after intravenous methylprednisolone therapy is begun, she begins to have visual hallucinations that her organ donor has entered her hospital room naked and is drawing symbols on her skin with an eyebrow pencil. Current medications include tacrolimus, mycophenolate mofetil, omeprazole, trimethoprim-sulfamethoxazole, acyclovir, and nystatin. The patient has no history of psychiatric illness. She is awake and alert. Physical examination shows a well-healing surgical incision over the abdomen. On mental status examination, she says she is worried about her visual hallucinations. She is oriented to person, place, and time. Which of the following is the most likely explanation for this patient's psychiatric symptoms?
- ☐ A) Adjustment disorder with anxious mood
 - ☐ B) Corticosteroid-induced psychotic disorder
 - ☐ C) Delirium
 - ☐ D) Malingering
 - ☐ E) Schizophrenia



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9. A 42-year-old man comes to the physician for a routine examination. He is accompanied by his wife. During the examination, his wife states that he is competitive, rigid, stubborn, and opinionated. She feels he has difficulty making simple decisions and discarding old items that are of no use and is anxious and obsessed with being early for everything. He is a successful lawyer, and his coworkers have said that he believes that his way is the only way of getting things done. When asked about this, he does not think there is a problem. Examination shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Generalized anxiety disorder
- ☐ B) Narcissistic personality disorder
- ☐ C) Obsessive-compulsive personality disorder
- ☐ D) Panic disorder
- ☐ E) Schizoid personality disorder

10. A 24-year-old man is brought to the emergency department from a psychiatric hospital because of a decreasing level of consciousness over the past 3 days. He has become progressively less responsive and has remained in bed for the past 12 hours without speaking or following commands. He has a history of schizophrenia with multiple psychiatric hospitalizations and is currently being treated with haloperidol and benztropine. His temperature is 39.4°C (103°F), pulse is 120/min, respirations are 28/min, and blood pressure is 180/100 mm Hg. He withdraws weakly to noxious stimuli. His eyes are open, but he does not fix or follow visual stimuli or gaze around the room. He is mute and does not follow commands. Examination shows marked rigidity of the neck and all extremities. Laboratory studies show:

Leukocyte count	18,000/mm ³
Segmented neutrophils	80%
Lymphocytes	20%
Serum creatine kinase	12,000 U/L
Cerebrospinal fluid	
Erythrocyte count	2/mm ³
Leukocyte count	1/mm ³
Protein	28 mg/dL
Glucose	78 mg/dL

A CT scan of the head shows no abnormalities. Which of the following is the most likely underlying cause?

- ☐ A) Catatonia
- ☐ B) Conversion reaction
- ☐ C) Dissociative fugue
- ☐ D) Drug reaction
- ☐ E) Infection
- ☐ F) Malingering
- ☐ G) Toxin

11. A 57-year-old woman comes to the physician because of difficulty sleeping, tearfulness, and restlessness since her daughter was diagnosed with metastatic breast cancer 3 days ago. She reports that when she goes to bed at night, she is unable to fall asleep for several hours and lays in bed worrying about her daughter's situation. The patient underwent a mastectomy for breast cancer 7 years ago. She takes acetaminophen/butalbital for occasional migraines. Her vital signs are within normal limits. Physical examination shows no abnormalities. On mental status examination, she is tearful and tense but calms during the conversation. There is no evidence of suicidal ideation. Which of the following is the most appropriate next step in management?

- ☐ A) Biofeedback
- ☐ B) Carbamazepine therapy
- ☐ C) Clonazepam therapy
- ☐ D) Clonidine therapy
- ☐ E) Imipramine therapy
- ☐ F) Olanzapine therapy
- ☐ G) Pentobarbital therapy
- ☐ H) Sertraline therapy



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12. A previously healthy 23-year-old man is brought to the emergency department by friends 20 minutes after the onset of bizarre behavior that began at a party. His friends say that he became paranoid and started threatening them. His temperature is 38.3°C (100.9°F). Physical examination shows agitation, diaphoresis, horizontal nystagmus, and occasional myoclonus. Mental status examination shows dysarthric speech and combativeness. His thoughts are disorganized, and he is unable to give a coherent history. He believes that the physician is trying to kill him. Thirty minutes later, he becomes somnolent. Administration of naloxone does not improve his condition. Which of the following is the most likely diagnosis?

- ☐ A) Alcohol intoxication
- ☐ B) Amphetamine intoxication
- ☐ C) Opioid intoxication
- ☐ D) PCP intoxication
- ☐ E) Schizophrenia



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13. A 37-year-old man comes to the physician because of increasing distress about his procrastination and indecision. He has been late for work often because he is easily distracted by any task; as a result, he has received poor job performance ratings. He arrived late for today's examination because he was awake late last night creating a new computer folder system and then had to refold the clothes in his closet several times this morning. He says he becomes nervous if he is unable to finish similar minor projects. He sometimes daydreams about projects and must complete them in his daydreams before attending to other tasks. He reports that his family is exasperated by his behavior. He has no history of serious illness and takes no medications. He appears fatigued. Physical examination shows no abnormalities. On mental status examination, he is fully oriented. He is cooperative but fidgets and has poor eye contact. He is able to repeat six digits forward, spell "world" backward, and perform serial sevens. He gives four interpretations of a proverb and is unable to decide on a final answer. Which of the following is the most likely diagnosis?

- ☐ A) Attention-deficit/hyperactivity disorder
- ☐ B) Delusional disorder
- ☐ C) Generalized anxiety disorder
- ☐ D) Obsessive-compulsive disorder
- ☐ E) Schizotypal personality disorder



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14. A 37-year-old man is brought to the emergency department by police after he was found wandering on the street. The officers say that the patient was screaming and talking to himself. He is unkempt and agitated, and he is wearing winter clothes on a hot summer day. Neurologic examination shows no focal findings. During the examination, he says that he is not going to take any medicine, and the "television told me about the murders." Which of the following is the most likely additional finding in this patient?

- ☐ A) Cerebral edema on CT scan of the head
- ☐ B) Enlarged lateral and third ventricles on CT scan of the head
- ☐ C) Increased serum dopamine concentration
- ☐ D) Increased serum and urine copper concentrations
- ☐ E) Temporal spikes on EEG



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15. A 42-year-old woman, gravida 1, para 1, comes to the physician because of a 5-month history of increasingly severe episodes of lower abdominal pain. The pain is exacerbated during bowel movements, sexual intercourse, and menses. Treatment with several analgesics has not controlled the pain. Laparoscopy 4 years ago and a second laparoscopy 2 years ago for evaluation of pain showed no abnormalities. She is sexually active with one male partner, and they use condoms for contraception. She is a single parent and lost her job 6 months ago. She now has difficulty sleeping 1 to 3 nights each week because she worries about finding a job. Abdominal examination shows tenderness in both lower quadrants. Pelvic examination shows a mobile, nontender uterus and normal, nontender adnexa. A complete blood count, erythrocyte sedimentation rate, Pap smear, and urine and cervical cultures show no abnormalities. Which of the following is the most appropriate next step to determine the cause of this patient's pain?

- ☐ A) MRI of the pelvis
- ☐ B) Psychiatric assessment
- ☐ C) Hysteroscopy
- ☐ D) Laparoscopy
- ☐ E) Endometrial biopsy



16. A 14-year-old girl is brought to the physician by her mother because of poor academic performance since her family relocated to a new town 6 months ago. The patient is halfway through the eighth grade and has been earning mostly D's and F's in school. She is having trouble making friends, especially since she lost her place on the school track team because of her poor academic performance. In her last school, she earned mostly A's and B's and had many friends. The family moved after the sudden death of the patient's father as a result of a motor vehicle collision in which the patient was a passenger. The patient had sustained minor injuries that were successfully treated. Since the family moved, she has been having frequent nightmares, is easily startled, and has trouble concentrating. Her mother says that her daughter seems to be "walking around in a cloud" much of the time. Physical examination shows no abnormalities. On mental status examination, the patient fidgets and is somewhat slow to respond to questions. She is oriented to person, place, and time. She says she feels nervous all of the time. Which of the following is the most likely diagnosis?

- ☐ A) Acute stress disorder
- ☐ B) Adjustment disorder
- ☐ C) Panic disorder
- ☐ D) Post-traumatic stress disorder
- ☐ E) Separation anxiety disorder



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17. A 27-year-old woman is brought to the emergency department 2 hours after cutting her arms superficially with a razor blade. On arrival, she appears calm and is holding a stuffed animal. She says that she cut herself because she wanted to "feel something" and is unsure if she will be able to stop herself from doing this again. She says that she sleeps 8 hours each night and has a good appetite. From the ages of 10 to 15 years, she was physically and sexually abused by her stepfather and mother on several occasions. She has never had a long-term relationship and has been fired from several jobs because of difficulty getting along with her supervisors. Her pulse is 80/min, respirations are 16/min, and blood pressure is 120/60 mm Hg. Physical examination shows lacerations over both forearms. Neurologic examination shows no abnormalities. On mental status examination, she denies depressed mood or suicidal ideation but is convinced that she is worthless and deserves to die. Which of the following is the most likely diagnosis?

- ☐ A) Borderline personality disorder
- ☐ B) Conversion disorder
- ☐ C) Dissociative identity disorder
- ☐ D) Hypochondriasis
- ☐ E) Major depressive disorder
- ☐ F) Malingering
- ☐ G) Post-traumatic stress disorder



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18. An 87-year-old man is admitted to the hospital because of dehydration and emaciation. He appears catatonic and is unresponsive to questions. One month ago, he began withdrawing from relatives and talking constantly about death. He gradually stopped eating and drinking and has had a 9-kg (20-lb) weight loss during this time. He has a 35-year history of major depressive disorder and has attempted suicide twice. He has no chronic medical conditions and currently takes no medications. He appears cachectic and is staring into space. He does not respond to verbal stimuli. He is 168 cm (5 ft 6 in) tall and weighs 50 kg (110 lb); BMI is 18 kg/m². Examination shows temporal wasting. Which of the following is the most appropriate next step in management?

- ☐ A) Biofeedback
- ☐ B) Desipramine therapy only
- ☐ C) Fluoxetine therapy only
- ☐ D) Desipramine and risperidone therapy
- ☐ E) Fluoxetine and risperidone therapy
- ☐ F) Electroconvulsive therapy



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19. A 25-year-old primigravid woman at 16 weeks' gestation has had increased episodes of crying, difficulty falling asleep, decreased energy, and loss of appetite over the past 6 weeks. She has a history of recurrent major depressive disorder. Her family is concerned that she might inadvertently harm herself or her fetus. Her husband reports that she has lost interest in daily activities. She states that she has no suicidal thoughts. She is concerned about her ability to take care of a newborn and is questioning her decision not to terminate the pregnancy. Which of the following is the most appropriate next step in management?

- ☐ A) Reassurance with return visit in 2 weeks
- ☐ B) Explanation of the risks and benefits of antidepressant therapy
- ☐ C) Explanation of the risks and benefits of therapeutic abortion
- ☐ D) Support group therapy
- ☐ E) Admission to the hospital



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20. A 17-year-old boy is brought to the emergency department by his mother because of a 20-minute episode of severe chest pain and sweating that started 1 hour ago while he was watching television. On arrival, he is alert and repeatedly expresses concern that something is "very wrong." He reports that during the episode, his heart was "pounding," he felt short of breath, and he was sure he was having a heart attack and was going to die. One month ago, he had a similar episode that occurred in the evening approximately 30 minutes after his high school football practice. At that time, results of laboratory studies and an ECG obtained in the emergency department showed no abnormalities. After the initial episode, he quit the football team, and he has continued to limit his physical activity because he is afraid of bringing on another episode. He reports no insomnia or changes in appetite. His mother states that he was diagnosed with an innocent heart murmur at the age of 6 months, but it has not been noted on examinations since the age of 1 year. His paternal grandfather died of a myocardial infarction 3 years ago at the age of 68 years. The patient takes no medications. He does not smoke cigarettes, drink alcohol, or use illicit drugs. He performs well in school. He is 180 cm (5 ft 11 in) tall and weighs 73 kg (161 lb); BMI is 23 kg/m². His temperature is 37°C (98.6°F), pulse is 98/min, respirations are 20/min, and blood pressure is 130/83 mm Hg. Cardiac examination shows a normal S₁ and S₂ with variation of S₂ on respiration. The remainder of the examination shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Acute stress disorder
- ☐ B) Delusional disorder
- ☐ C) Generalized anxiety disorder
- ☐ D) Hypertrophic cardiomyopathy
- ☐ E) Idiopathic recurrent pericarditis
- ☐ F) Panic disorder
- ☐ G) Paroxysmal supraventricular tachycardia



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21. A 30-year-old law school graduate is caught cheating on the bar examination. Further investigation shows previous allegations of cheating in junior high school and an adolescence characterized by occasional truancy, frequent fights, and lying. As an adult, he has not been able to hold a job, has been bankrupt once, and was divorced because of failure to provide proper care for his children. He is angry about the accusation of cheating and insists that the incident is a misunderstanding. He has many friends. Which of the following is the most likely diagnosis?

- ☐ A) Antisocial personality disorder
- ☐ B) Borderline personality disorder
- ☐ C) Dissociative fugue
- ☐ D) Dysthymic disorder
- ☐ E) Narcissistic personality disorder



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22. A 37-year-old male police officer comes to the physician at the request of his superiors 1 week after he witnessed a terrorist bombing during which several civilians and three fellow police officers were killed. He sustained only minor injuries and assisted in rescuing survivors and gathering body parts. Since the bombing, he has felt emotionally numb and has been unable to enjoy activities he used to find pleasurable. He has continued to work but has requested assignments far removed from the site of the attack. He describes his sleep as fitful. Two years ago, he sustained a bullet wound to the right calf, which left him with a slight limp. He has no other history of medical or psychiatric illness. He says he used to drink one to two beers nightly but has been drinking two to three beers nightly for the past week. Physical examination confirms the previous gunshot injury to the calf and shows no other abnormalities. On mental status examination, he is irritable and says he is not depressed. He tells the physician, "They made me come. I'm not interested in talking with anyone. I just want to spend time with my buddies and be left alone." Which of the following is the most appropriate initial step in management?

- ☐ A) Encourage the patient to discuss the trauma in detail
- ☐ B) Provide information about the range of reactions to trauma
- ☐ C) Recommend group therapy with other trauma survivors
- ☐ D) Recommend a 1-month medical leave of absence from active duty
- ☐ E) Recommend a physician-led trauma debriefing series
- ☐ F) Recommend a 12-step program
- ☐ G) Begin clonazepam therapy
- ☐ H) Begin fluoxetine therapy



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23. A previously healthy 27-year-old man is brought to the emergency department by his girlfriend because of a 6-week history of daily episodes of hearing a voice telling him that he is Christ. He is convinced that he has the power to heal the sick and requests that he be admitted into the wards to help the patients. During the past 4 weeks, he has had a 10-kg (22-lb) weight loss. He has not slept more than 2 hours nightly for 2 weeks. When he is awake at night, he watches television or calls strangers on the telephone. He does not drink alcohol or use illicit drugs. He is 173 cm (5 ft 8 in) tall and weighs 80 kg (176 lb); BMI is 27 kg/m². His temperature is 37°C (98.6°F), pulse is 110/min, and blood pressure is 160/90 mm Hg. Physical examination shows no other abnormalities. On mental status examination, he appears restless and agitated. He flirts with female staff members and tries to convince people of his powers. He has pressured speech and talks incessantly. There is no evidence of visual hallucinations. Recall is difficult to assess because he cannot concentrate. Long-term memory is intact. He is oriented to person, place, and time. Which of the following is the most likely diagnosis?

- ☐ A) Bipolar disorder
- ☐ B) Cyclothymic disorder
- ☐ C) Major depressive disorder with psychotic features
- ☐ D) Schizoaffective disorder
- ☐ E) Schizophrenia



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24. A 5-year-old boy has had temperatures to 38.9°C (102°F), cough, and coryza for 2 days and visual hallucinations of animals and people for 2 hours. His mother has been giving him an over-the-counter cold medicine for 24 hours, but it has not relieved his symptoms. His temperature is 38.8°C (101.8°F). Examination shows a clear nasal discharge. Which of the following is the most likely reason for the child's hallucinations?

- ☐ A) Behavioral abnormalities
- ☐ B) Drug toxicity
- ☐ C) Hyperpyrexia
- ☐ D) Increased intracranial pressure
- ☐ E) Neuronal inflammation



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25. A 27-year-old man is brought to the emergency department 30 minutes after his brother found him agitated, tremulous, and complaining of a headache. He had eaten a peanut butter sandwich 12 hours before his symptoms began. He has a 10-year history of severe major depressive disorder. During this period, he has attempted suicide three times despite treatment with both selective serotonin reuptake inhibitors and tricyclic antidepressants. Eight days ago, he went to a new physician because of increasing despondency. This physician immediately discontinued the fluoxetine and substituted tranylcypromine to start 1 week later. He also instructed the patient to start a monoamine oxidase inhibitor diet immediately. Current medications also include diphenhydramine as needed for seasonal allergies. He has no history of hypertension, diabetes mellitus, or migraine. On arrival, he is agitated. His temperature is 40.6°C (105°F), pulse is 130/min, respirations are 26/min, and blood pressure is 180/120 mm Hg. Physical examination shows diaphoresis and tremor of the upper extremities. Deep tendon reflexes are 3+ bilaterally; there are no pathologic reflexes. On mental status examination, he is complaining of occipital headache. He is oriented to person but not to place or time. Which of the following is the most likely cause of these findings?

- ☐ A) Acute anxiety attack
- ☐ B) Acute fluoxetine withdrawal
- ☐ C) Drug-drug interaction
- ☐ D) Ingestion of peanut butter
- ☐ E) Use of diphenhydramine



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26. A previously healthy 27-year-old woman comes to the physician because of a 2-month history of crying spells, poor concentration, fatigue, and lethargy. She says that she sleeps at least 12 hours daily and is no longer interested in socializing. She recently broke up with her boyfriend of 10 months, and her work performance has begun to decline; she is afraid that she will lose her job if it does not improve. She does not drink alcohol or use illicit drugs. Menses occur at regular 28- to 30-day intervals. Her last menstrual period was 2 weeks ago. She has had a 9-kg (20-lb) weight gain over the past 9 months. She is 168 cm (5 ft 6 in) tall and weighs 66 kg (145 lb); BMI is 23 kg/m². Physical examination shows no abnormalities. Mental status examination shows a sad mood and restricted affect. Which of the following is the most likely diagnosis?

- ☐ A) Adjustment disorder with depressed mood
- ☐ B) Dysthymic disorder
- ☐ C) Generalized anxiety disorder
- ☐ D) Major depressive disorder
- ☐ E) Premenstrual dysphoric disorder



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27. A 15-year-old boy is brought to the emergency department by his two friends because of unusual behavior that began at a party 1 hour ago. His friends say that he became raucous and made an unwanted sexual advance to a girl he had just met. He then ran into the street, careless to oncoming cars, saying he was able to avoid them. His pulse is 84/min, respirations are 12/min, and blood pressure is 110/70 mm Hg. He is agitated when left alone, but he calms down when spoken to by nursing staff. Physical examination shows an unsteady gait. Pupils are equal and reactive. Which of the following is the most likely drug taken?

- ☐ A) Alcohol
- ☐ B) Amphetamine
- ☐ C) Cocaine
- ☐ D) Heroin
- ☐ E) LSD
- ☐ F) Marijuana



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28. An 8-year-old boy is brought for a school physical examination. He was diagnosed with attention-deficit/hyperactivity disorder 2 years ago and was treated with methylphenidate therapy for 3 months with moderate symptom resolution. Methylphenidate was discontinued because of persistent loss of appetite and weight loss. Desipramine therapy was started at 25 mg daily and gradually increased to 100 mg daily. His symptoms have improved, and he has had no adverse effects. An ECG shows a QTc of 0.5 sec ($N \leq 0.44$). Which of the following is the most appropriate initial step in management?

- ☐ A) 24-Hour ambulatory ECG monitoring
- ☐ B) Echocardiography
- ☐ C) Discontinue desipramine therapy
- ☐ D) Increase desipramine dose
- ☐ E) Switch to bupropion therapy



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29. A 67-year-old man is brought to the physician by his wife because of a 1-month history of increasingly "paranoid" behavior. He is convinced that his wife is trying to kill him. His wife reports that he has been reluctant to eat food unless the containers are opened in front of him. He has Parkinson disease, which has been well controlled with carbidopa-levodopa for 4 years. Trials of reducing the dose have resulted in an inability to walk because of significant worsening of his gait. He takes no other medications. His temperature is 37°C (98.6°F), pulse is 83/min, respirations are 19/min, and blood pressure is 125/86 mm Hg. Neurologic examination shows mild bradykinesia and diffuse rigidity; no tremor is noted. There is mild involuntary ankle rotation bilaterally. On mental status examination, he is calm and oriented to person and place but not to time or date. He has a low mood and blunted affect. He is reluctant to speak with his wife present. When alone, he briefly indicates that his wife is not to be trusted and will not give further details. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Gabapentin
- ☐ B) Haloperidol
- ☐ C) Modafinil
- ☐ D) Quetiapine
- ☐ E) Valproic acid



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30. A previously healthy 57-year-old woman comes to the physician with her husband because of an 8-week history of progressive confusion and memory loss. She is no longer able to work at her job as a public relations manager because she has become so withdrawn. Her husband says that she was previously outgoing and talkative but now hardly says a word and is unable to comprehend simple conversations. Her speech is dysarthric. Examination shows diffuse hyperreflexia and prominent snout, grasp, and palmomental reflexes. Babinski sign is present bilaterally. She exhibits startle myoclonus to loud noises. Her Mini-Mental State Examination score is 8/30. Laboratory studies, including cerebrospinal fluid analysis, are within normal limits. An MRI of the brain shows no abnormalities. EEG shows generalized slowing with periodic sharp waves. Which of the following is the most likely diagnosis?

- ☐ A) Creutzfeldt-Jakob disease
- ☐ B) Dementia, Alzheimer type
- ☐ C) Multiple sclerosis
- ☐ D) Parkinson disease
- ☐ E) Pick disease



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31. An 82-year-old man with dementia, Alzheimer type, is brought to the physician because of a 4-day history of visual hallucinations and irritability. He has a 2-month history of insomnia. He has osteoarthritis, hypertension, and hypothyroidism. His medications are rivastigmine, ibuprofen, atenolol, levothyroxine, and amitriptyline. His blood pressure is 128/88 mm Hg. Physical examination shows no abnormalities. On mental status examination, he recalls zero of three objects after 5 minutes. During the examination, he points his finger and sternly says, "You need to stop doing that right this minute." When asked whom he is addressing, he says, "that tiny little man shouting over in the corner." Results of a complete blood count, measurement of serum glucose concentration, and kidney and liver function tests are within the reference ranges. Urinalysis shows no abnormalities. Discontinuation of which of the following of this patient's medications is most appropriate at this time?

- ☐ A) Amitriptyline
- ☐ B) Atenolol
- ☐ C) Ibuprofen
- ☐ D) Levothyroxine
- ☐ E) Rivastigmine



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32. A 17-year-old girl is brought to the physician by her parents because of a 20-kg (45-lb) weight loss during the past 6 months. She also has become more isolated from her friends and has lost interest in activities she used to enjoy. Her academic performance has deteriorated from excellent to average. She has major depressive disorder treated with fluoxetine for 3 years. She recently stopped taking it because of concern that it was preventing her from losing weight. She is 160 cm (5 ft 3 in) tall and weighs 41 kg (90 lb); BMI is 16 kg/m². Her pulse is 46/min, and palpable systolic blood pressure is 86 mm Hg. On mental status examination, she says she has been feeling poorly lately but has attributed it to stress at school and pressure from her parents to complete college applications. She is pleased that she has lost weight but feels that she has "more to lose." She says her appetite has not decreased and that she feels hungry a good part of the time. There is no evidence of suicidal ideation. She is adamantly opposed to beginning fluoxetine again. She is not interested in counseling and does not think there is "anything wrong" with her. Her parents want her to be admitted to a psychiatric hospital, but she is opposed. Which of the following is the most appropriate next step?

- ☐ A) Begin outpatient psychotherapy
- ☐ B) Recommend family counseling
- ☐ C) Begin bupropion therapy
- ☐ D) Begin parenteral nutrition
- ☐ E) Admit her to a psychiatric hospital



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The response options for the next **2** items are the same. Select one answer for each item in the set.

For each patient with anxiety symptoms, select the most appropriate pharmacotherapy.

- ☐ A) Bupropion
- ☐ B) Chlorpromazine
- ☐ C) Fluoxetine
- ☐ D) Haloperidol
- ☐ E) Imipramine
- ☐ F) Lorazepam
- ☐ G) Perphenazine
- ☐ H) Propranolol

33. A 47-year-old man has had tension and hand tremors for 1 month. He has been treated for bipolar disorder with lithium carbonate for 6 months. He has a 2-year history of alcohol abuse on weekends. His temperature is 36.7°C (98°F), pulse is 90/min, respirations are 16/min, and blood pressure is 140/86 mm Hg. Examination shows mild tremors of the fingers and hands. Serum lithium carbonate concentration is 1 mEq/L (therapeutic range=0.6–1.2). Serum electrolyte concentrations and thyroid function tests are within normal limits.



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For each patient with anxiety symptoms, select the most appropriate pharmacotherapy.

- ☐ A) Bupropion
- ☐ B) Chlorpromazine
- ☐ C) Fluoxetine
- ☐ D) Haloperidol
- ☐ E) Imipramine
- ☐ F) Lorazepam
- ☐ G) Perphenazine
- ☐ H) Propranolol

34. An otherwise healthy 25-year-old man comes to the physician because he has a severe fear of flying and must attend an essential business meeting in Europe in 2 days. He has no substance abuse problems or other mental or physical problems.



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The response options for the next **2** items are the same. Select one answer for each item in the set.

For each patient with a change in behavior, select the most likely affected region of the brain.

- ☐ A) Amygdaloid nucleus
- ☐ B) Caudate nucleus
- ☐ C) Medial geniculate nucleus
- ☐ D) Nucleus basalis of Meynert
- ☐ E) Red nucleus
- ☐ F) Substantia nigra
- ☐ G) Subthalamic nucleus

35. A 47-year-old man with Down syndrome is brought to the physician because of increasing forgetfulness and irritability over the past 10 months. He has been awakening at 3 am to get dressed for the day and now gets lost when he takes his daily walk. He appears disheveled. Examination shows decreased memory and a normal mood and affect. During the interview, he attempts to leave the office, saying "take me home."



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For each patient with a change in behavior, select the most likely affected region of the brain.

- ☐ A) Amygdaloid nucleus
- ☐ B) Caudate nucleus
- ☐ C) Medial geniculate nucleus
- ☐ D) Nucleus basalis of Meynert
- ☐ E) Red nucleus
- ☐ F) Substantia nigra
- ☐ G) Subthalamic nucleus

36. A 37-year-old man is brought to the physician because of a change in personality over the past 4 months. He has become sexually disinhibited, tactless, impulsive, and easily irritated. His father had similar symptoms at the age of 42 years. Examination shows involuntary movements of the tongue. He has a sudden coarse involuntary jerking movement of the upper extremity that he attempts to hide by pretending to straighten his hair.



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37. A 25-year-old woman comes to the physician at her husband's request because of increasing irritability and tense mood over the past year. Her husband would like to start a family, and he is concerned about her ability to handle the added responsibility. She says that she worries about their finances despite the fact that they have no debt and she and her husband have jobs that pay well. Her pulse is 84/min, and blood pressure is 120/70 mm Hg. Physical examination shows no abnormalities except for sweaty palms. Mental status examination shows tension and anxiety. She admits to occasional difficulty sleeping and periods of mild depression but does not think that this is out of the ordinary. Laboratory studies show:

Hematocrit	38%
Erythrocyte count	4 million/mm ³
Leukocyte count	5500/mm ³
Serum	
Thyroid-stimulating hormone	0.5 μU/mL
Thyroxine (T ₄)	12 μg/dL

Toxicology screening is negative. Which of the following is the most appropriate pharmacotherapy for this patient?

- ☐ A) Bupropion
- ☐ B) Buspirone
- ☐ C) Levothyroxine
- ☐ D) Methimazole
- ☐ E) Quetiapine

38. A 22-year-old woman is brought to the emergency department because of a 4-hour history of violent, agitated behavior. Her college roommates are concerned that she may have used illicit drugs. She has no known history of similar behavior or serious illness. She takes no medications. Her temperature is 38°C (100.4°F), pulse is 120/min, respirations are 20/min, and blood pressure is 140/90 mm Hg. On mental status examination, her speech is loud and rapid in rate and rhythm. She describes her mood as "wild"; her affect is reactive but primarily exuberant and irritable. Her thought process is tangential. She reports no hallucinations. During the examination, she tries to leave the emergency department, pushing a security guard aside and throwing a clipboard on the floor. Intoxication with which of the following substances is the most likely cause of this patient's symptoms?

- ☐ A) Cannabis
- ☐ B) Ecstasy (3,4-methylenedioxymethamphetamine)
- ☐ C) Heroin
- ☐ D) LSD
- ☐ E) Methamphetamine



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39. A 13-year-old girl is brought to the physician by her father for a well-child examination. He is concerned about her weight and eating habits. He says that she is always "on the go" and never sits down for a full meal. She will often just grab a piece of fruit when she is on her way to an activity. She will eat a full meal when the family has dinner together every Sunday. She has talked about wanting to become a vegetarian because of her concern for animals. She has had the same group of friends since elementary school. She is the captain of her soccer team and practices 4 days weekly. She is active in an after-school drama program 3 days weekly and is the lead in this season's play. She maintains a B grade average. Her father says that she talks on the telephone "constantly" and is animated and cheerful most of the time. When she is in her bedroom, she prefers to keep the door closed and stops talking if she is on the telephone and her father walks into the room. She has a disorganized bedroom and often falls fast asleep on a bed covered with piles of clothes. Menarche has not occurred. She appears thin and muscular. She is at the 50th percentile for height and 35th percentile for weight, which is unchanged from last year. Vital signs are within normal limits. Breast and pubic hair development are Tanner stage 2. Physical examination shows no abnormalities. When interviewed alone, she is animated. She shrugs and rolls her eyes when asked about her diet and weight. She thinks she is fine and does not understand why her father is so upset. Which of the following is the most appropriate next step?
- ☐ A) Reassure the father that this is normal development
 - ☐ B) Recommend individual psychotherapy
 - ☐ C) Recommend nutritional counseling
 - ☐ D) Recommend that the father keep a log of what his daughter eats
 - ☐ E) Schedule weekly examination and weighing



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40. A 3-year-old girl is brought to the emergency department 1 hour after injuring her right arm. This is her third visit to the emergency department in 6 months. The parents say that she fell. The child is too upset to say how she injured herself. Her parents note that she is overly aggressive in day care, where she is not responding to instructions, and has hit other children. During her previous visit for a wrist injury, an x-ray of the wrist showed normal findings. On examination, the child appears fearful of the examiner, provides limited responses, and is restless and easily distracted. An x-ray of the right humerus shows a closed fracture. Which of the following is the most appropriate next step in management?

- ☐ A) Admit to hospital
- ☐ B) Cognitive behavior therapy
- ☐ C) Contact child protective services
- ☐ D) CT scan of the head
- ☐ E) Family counseling
- ☐ F) Follow-up outpatient appointment in 2 weeks
- ☐ G) Neuropsychological testing
- ☐ H) Psychiatric assessment



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41. An 8-year-old boy is brought to the physician because of a 1-year history of increasingly frequent episodes of eye blinking and facial grimacing during the past 6 months that now occur several times daily. He also frequently makes grunting and throat-clearing noises. His teacher often sends him out of the room for being disruptive. He says that he will repeatedly shrug one of his shoulders and attempt to hide this behavior by smoothing his hair. He is embarrassed by these behaviors and can suppress them with effort, but they often return when he is distracted and are exacerbated by stress. The behaviors do not occur during sleep. He has no history of serious illness and takes no medications. During the examination, he has several episodes of rapid, forceful eye blinking and throat clearing. Physical examination shows no other abnormalities. Mental status examination shows a mildly anxious mood and affect. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Dextroamphetamine
- ☐ B) Hydroxyzine
- ☐ C) Imipramine
- ☐ D) Lithium carbonate
- ☐ E) Methylphenidate
- ☐ F) Risperidone



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42. A 67-year-old woman, whose husband died 18 months ago, is admitted to the hospital because of severe headaches for 4 weeks. She is quiet and seems sad. She believes that she has brain cancer, deserves to be punished for her sins, and is being punished by God for past misdeeds. She has had fatigue and a 6.8-kg (15-lb) weight loss over the past 4 months. Physical and neurologic examinations show no abnormalities. Which of the following is the most likely diagnosis?
- ☐ A) Adjustment disorder with mixed disturbance of emotions and conduct
 - ☐ B) Delusional disorder
 - ☐ C) Major depressive disorder
 - ☐ D) Schizophrenia
 - ☐ E) Somatization disorder



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43. A 23-year-old man is brought to the physician by his mother because of increasing conflict between the two of them over the past 6 months. She says that he has become more difficult because of his growing concern about neighbors who he believes abuse the welfare system. He says that he has had special abilities to predict the future since the age of 15 years, but no one in his family believes him. His mother says that he has never had many friends and describes him as a loner. He has not used any drugs. He has not felt sad or depressed and has had no recent change in appetite or sleep disturbances. Physical examination shows no abnormalities. His language is vague and stilted. Which of the following is the most likely personality disorder in this patient?

- ☐ A) Antisocial
- ☐ B) Avoidant
- ☐ C) Borderline
- ☐ D) Histrionic
- ☐ E) Narcissistic
- ☐ F) Obsessive-compulsive
- ☐ G) Paranoid
- ☐ H) Schizoid
- ☐ I) Schizotypal



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44. A 47-year-old woman comes to the physician because she has had difficulty sleeping at night since her youngest daughter left for college 2 months ago. During the past week, she has been struggling to stay awake at work. She drinks two to three cocktails to help her fall asleep and 3 to 4 oz of vodka in the morning to help calm her nerves. She is often anxious and has frequent heartburn. She was told by her physician that the heartburn is likely caused by her alcohol consumption. She does not want to alter her routine because she has become increasingly anxious and irritable when she has tried to do so. Her temperature is 37.2°C (99°F), pulse is 90/min, respirations are 12/min, and blood pressure is 150/90 mm Hg. Physical examination shows no other abnormalities. On mental status examination, she is mildly anxious but polite and friendly. She says that she is not depressed but that it was difficult for her to adjust when her daughter first left for college. Laboratory studies show:

Erythrocyte count	3500/mm ³
Mean corpuscular volume	102 μm ³
Leukocyte count	4500/mm ³
Serum	
Mg ²⁺	1.5 mEq/L
Thyroid-stimulating hormone	3.5 μU/mL
AST	40 U/L
ALT	25 U/L

An ECG shows a normal sinus rhythm. Which of the following is the most likely diagnosis?

- ☐ A) Adjustment disorder
- ☐ B) Alcohol dependence
- ☐ C) Circadian rhythm sleep disorder
- ☐ D) Generalized anxiety disorder
- ☐ E) Major depressive disorder
- ☐ F) Primary insomnia

45. A homeless 27-year-old man who is being treated with antipsychotic and antiparkinsonian drugs comes to the emergency department because he feels anxious and has difficulty urinating. He reports seeing visions. He is hyperactive and disoriented to time and place and has difficulty focusing on nearby objects. Physical examination shows dry mouth, flushed skin, and tachycardia at rest. There is no evidence of muscular rigidity. Which of the following is the most likely diagnosis?
- ☐ A) Delirium due to anticholinergic medication
 - ☐ B) Extrapyrarnidal syndrome
 - ☐ C) Neuroleptic malignant syndrome
 - ☐ D) Noncompliance with antipsychotic drug therapy
 - ☐ E) Psychosis due to substance abuse



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46. A 24-year-old woman comes to the physician with her husband because of an 8-year history of increasing daytime fatigue. She says that she has difficulty remaining awake whenever she sits down to relax. She has never been able to drive more than 30 minutes without becoming so sleepy that she needs to pull the car over and stop. Her husband says that once every 2 months, she gets up in the middle of the night and does not respond when he calls her name. He says that she does not snore. The patient reports that in high school she trained herself not to laugh because she felt weak and could not remain standing after her friends told jokes. She has no history of serious illness. Her only medication is an oral contraceptive. Physical examination shows no abnormalities. On mental status examination, she appears concerned and has a reactive affect. She is alert and oriented to person, place, and time. Which of the following is most likely to confirm the diagnosis?

- ☐ A) Keeping a sleep log for 2 weeks
- ☐ B) MRI of the brain
- ☐ C) EEG
- ☐ D) Electromyography
- ☐ E) Polysomnography



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47. A 32-year-old woman with schizoaffective disorder is brought to the emergency department because of a 7-day history of progressive lethargy, loss of appetite, nausea, and leg swelling; she has had jaundice since this morning. Five weeks ago, she was hospitalized for treatment of her psychiatric symptoms. Treatment with three medications was started during hospitalization, but she does not remember their names. She has no history of liver disease or other serious illness. She appears drowsy. Her pulse is 72/min, and blood pressure is 105/70 mm Hg. There is scleral icterus. Abdominal examination shows tenderness in the right upper quadrant. Neurologic examination shows a fine bilateral hand tremor. Mental status examination shows no evidence of hallucinations or delusions. She is oriented to person, place, and year but not to the day of the week or month. Serum studies show:

Bilirubin, total	5 mg/dL
Alkaline phosphatase	160 U/L
AST	240 U/L
ALT	210 U/L

Which of the following medications is most likely responsible for these findings?

- ☐ A) Clozapine
- ☐ B) Haloperidol
- ☐ C) Lithium carbonate
- ☐ D) Trazodone
- ☐ E) Valproic acid



48. A 29-year-old man comes to the physician because of the inability to maintain an erection through completion of sexual activity. He has been married for 3 months and has not been able to complete coitus over the past 2 months. He is usually able to attain an erection by direct penile stimulation and is often aware of nocturnal erections when waking during the night. For the past month, he and his wife have attempted to have intercourse about twice a week. He drinks several alcoholic beverages before engaging in sexual activity in an effort to relax. He has had two major depressive episodes over the past 4 years and was successfully treated with desipramine; he has not taken the medication since his marriage. He has borderline hypertension but takes no medications for it. He has a strong family history of type 2 diabetes mellitus. He takes ranitidine for stomach problems. His vital signs are normal. Cranial nerve function appears normal. He walks normally. Laboratory studies show:

Hemoglobin	11 g/dL
Platelet count	178,000/mm ³
Serum	
Fasting glucose	86 mg/dL
γ-Glutamyltransferase	70 U/L (N=5–50)

Which of the following is the most likely explanation for these findings?

- ☐ A) Alcohol use
- ☐ B) Desipramine therapy
- ☐ C) Diabetes mellitus
- ☐ D) Generalized anxiety disorder
- ☐ E) Major depressive disorder
- ☐ F) Marital conflict
- ☐ G) Ranitidine therapy

49. A 7-year-old boy is brought to the physician by his adoptive parents because of ongoing concern about his behavior and academic performance. He is two grade levels behind his peers in reading and frequently has fights with other children. He has no history of serious illness and takes no medications. His biological mother abused multiple substances before and during pregnancy. On adoption at the age of 15 months, there was some evidence that she may have physically abused him. He is at the 10th percentile for height and 5th percentile for weight. His pulse is 82/min, and blood pressure is 116/78 mm Hg. Examination shows a flattened nasal bridge, a long philtrum, and prominent ears. During the interview, he is friendly but becomes frustrated and distracted and is unable to sit still. Which of the following is the most likely diagnosis?

- ☐ A) Angelman syndrome
- ☐ B) Fetal alcohol syndrome
- ☐ C) Fragile X syndrome
- ☐ D) Rett disorder
- ☐ E) Shaken baby syndrome



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50. A 32-year-old woman comes to the physician because of increasingly severe pain that originates in her left shoulder and radiates to her elbow. She describes the pain as constant and burning, rating her current pain as a 7 on a 10-point scale. Eighteen months ago, she sustained a nerve injury of the left upper extremity in a motor vehicle collision. Since that time, she has been unable to return to work. Current medications include oxycodone and gabapentin. Physical examination shows atrophy of the left thenar eminence. Muscle strength in the left forearm and finger flexors is 3/5. On sensory examination, there is severe pain with light stroking of the anterolateral aspect of the left arm. Further sensory testing is deferred. During the examination she tells her physician, "I'm tired of all this. My medication is not strong enough. It only takes the edge off my pain, which is only getting worse. I'm realizing I'll be like this forever." Which of the following is the most appropriate response by the physician?

- ☐ A) "Are you worried about more nerve damage developing?"
- ☐ B) "Do you ever use more pain medication than is prescribed?"
- ☐ C) "Have you been feeling like just giving up?"
- ☐ D) "Is the pain caused by touch socially limiting?"
- ☐ E) "Let's review your medical concerns."



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